

Defendants filed a Rely on April 6, 2026.

Analysis

When awarding costs, a court must apply the following principles: (1) costs are allowable if incurred, whether or not paid; (2) allowable costs must be reasonably necessary to the conduct of the litigation, not merely convenient or beneficial to its preparation, and (3) allowable costs must be reasonable in amount. See CJER, *California Judges Benchbook: Civil Proceedings—Trial* (2025) ("CJER Civ. Pro.—Trial"), § 16.17; see Code Civ. Proc. § 1033.5. If the items on a verified memorandum of costs appear to be proper charges, the memorandum is prima facie evidence of their propriety and the burden is on the party contesting them to show that they were not reasonable or necessary. *Hooked Media Group, Inc. v. Apple Inc.* (2020) 55 Cal.App.5th 323, 338; see CJER Civ. Pro.—Trial, § 16.18.

Disposition

The Court finds and orders as follows:

1. The **PARTIES ARE ORDERED TO APPEAR.**

19. 9:00 AM CASE NUMBER: MSC21-01664
CASE NAME: DUGGAN VS. BDM ASSOCIATION
***HEARING ON MOTION IN RE: BIFURCATE**
FILED BY: BDM ASSOCIATION, INC, A CALIFORNIA NONPROFIT MUTUAL BENEFIT CORPORATION
TENTATIVE RULING:

Denied as moot given the parties' stipulation and Court's order regarding bifurcation.

20. 9:00 AM CASE NUMBER: MSC21-02668
CASE NAME: RENEE COTTON VS. MONICA MARSHALL
HEARING ON SUMMARY MOTION
FILED BY: GHC OF WALNUT CREEK, LLC.
TENTATIVE RULING:

Before the Court is a motion for summary judgment filed by defendants GHC of Walnut Creek, LLC and Life Generations Healthcare LLC. For the reasons set forth, the motion is **denied**.

Background

Plaintiff Renee Cotton alleges her claims individually "and/or as the surviving heir and/or dependent" of Dorothy Marshall, the decedent, "and/or as personal representative and successor in interest" of Dorothy Marshall ("Marshall" or "Decedent"). (First Amended Complaint ("FAC") ¶ 2.) The FAC, the operative complaint in the case, alleges three causes of action for professional negligence, elder abuse, and wrongful death. The Court sustained a demurrer to the FAC by GHC of Walnut Creek, LLC and Life Generations Healthcare LLC to the first and second causes of action against them for professional negligence and elder abuse and neglect under the Elder Abuse and Dependent Adult Civil Protection Act, Welfare & Institutions Code section 15600, *et seq.* ("Elder Abuse Act"). There remains

one cause of action for wrongful death against defendants GHC and Life Generations. (Def. Separate Statement ("SS") of Undisputed Material Facts ("UMFs"), UMFs 2, 5, 6; Pl. Resp. to UMFs 2, 5, 6.)

Generally, Plaintiff alleges that Dorothy Marshall, who was approximately 85, had a stroke on September 11, 2020 and cardiac arrest in January 2021. (FAC ¶ 13.) Thereafter, she was admitted to various hospitals and other healthcare facilities for care and treatment, including to a GHC facility in Walnut Creek from January 27, 2021 to February 9, 2021 (14 days), and from February 13, 2021 to March 20, 2021 (35 days). Dorothy Marshall died more than eight months later on November 27, 2021. (FAC ¶ 22.)

GHC and Life Generations move for summary judgment. Plaintiff opposes the motion.

Legal Standards for Ruling on Defendants' Motion

Under Code of Civil Procedure section 437c(c), a motion for summary judgment "shall be granted if all the papers submitted show that there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law." (Code Civ. Proc. § 437c(c).) When a defendant moves for summary judgment, the defendant bears the burden of producing evidence that plaintiff cannot prove one or more essential elements of plaintiff's claim or that there is a complete defense to the claim. (*Aguilar v. Atlantic Richfield Co.* (2001) 25 Cal.4th 826, 850; Code Civ. Proc. § 437c(p)(2).) Once the defendant meets this initial burden, the burden shifts to the plaintiff to produce admissible evidence showing that a triable issue of fact exists. (*Advent, Inc. v. National Union Fire Ins. Co. of Pittsburgh, PA* (2016) 6 Cal.App.5th 443, 453; Code Civ. Proc. § 437c(p)(2).)

The issues to be considered on a motion for summary judgment are defined by the pleadings. (*Doe v. Good Samaritan Hospital* (2018) 23 Cal.App.5th 653, 661.) The Court may not weigh the evidence but must view it "in the light most favorable to the opposing party and draw all reasonable inferences in favor of that party. [Citations omitted.]" (*Weiss v. People ex rel. Dept. of Transportation* (2020) 9 Cal.5th 840, 864.) Declarations and other evidence offered in support of a motion for summary judgment are strictly construed, while declarations and evidence offered in opposition to the motion are liberally construed. (*D'Amico v. Bd. of Medical Examiners* (1974) 11 Cal.3d 1, 20; *Johnson v. American Standard* (2008) 43 Cal.4th 56, 64.)

" '[S]ummary judgment cannot be granted when the facts are susceptible to more than one reasonable inference . . . ' " (*Husman v. Toyota Motor Credit Corp.* (2017) 12 Cal.App.5th 1168, 1180; *Cohen v. Five Brooks Stable* (2008) 159 Cal.App.4th 1476, 1497.) "All doubts as to the propriety of granting the motion—i.e., whether there is any triable issue of material fact—are to be resolved in favor of the party opposing the motion." (*Cohen v. Five Brooks Stable, supra*, 159 Cal.App.4th at 1483.)

Request for Judicial Notice

Moving parties request that the Court take judicial notice of the initial complaint, the copy of Ms. Marshall's death certificate, and the FAC. The Court **denies** the request as to the initial complaint, as that complaint has been superseded by the FAC and has no relevance to the issues subject to the motion. (*Jordache Enterprises, Inc. v. Brobeck, Phleger & Harrison* (1998) 18 Cal.4th 739, 748, fn. 6 [judicial notice denied where "the requests present no issue for which judicial notice of these items is necessary, helpful, or relevant"].) The Court **grants** the request as to the death certificate and the

FAC, subject to the limitations on judicial notice stated in the case law. (*See, e.g., StorMedia Inc. v. Superior Court* (1999) 20 Cal.4th 449, 457, fn. 9 [existence of document judicially noticeable, but not the truth of its contents or its proper interpretation where the truth is disputable, or the contents subject to interpretation]; *Fremont Indemnity Co. v. Fremont General Corp.* (2007) 148 Cal.App.4th 97, 113-115 [same].)

Procedural Notes

A. Reply Separate Statement

Code of Civil Procedure section 437c as amended effective January 1, 2025 states in relevant part, "The reply shall not include any new evidentiary matter, additional material facts, or separate statement submitted with the reply and not presented in the moving papers or opposing papers." (Code Civ. Proc. § 437c(b)(4) [emphasis added].) Even if Moving Parties' reply separate statement does not include additional material facts or new evidentiary matter, it includes extensive argument that, if considered, would violate the letter and spirit of the reply page limitations on a summary judgment motion. Further, Code of Civil Procedure section 437c, read as a whole, together with California Rule of Court, Rule 3.1350, do not indicate that argument, rather than facts and evidence, is properly included in a separate statement. The Court therefore does not consider the reply separate statement.

B. Evidentiary Objection in Plaintiff's Response to UMF 123

Plaintiff's response to UMF 123 states an evidentiary objection to a statement in Ms. Dennin's declaration in support of the motion, which does not comply with Rule 3.1354 of the California Rules of Court applicable to summary judgment motions. The Court's ruling does not consider that objection.

Analysis

The parties do not dispute that "Plaintiff's wrongful death claim is founded on the 'wrongful acts' to which Decedent was subjected while in the Defendants' care and custody which are the basis of Plaintiff's Elder Abuse Act claim." (Opp. p. 15, ll. 16-18.) The FAC supports that the "wrongful acts" alleged in the Elder Abuse Act cause of action are the basis for the wrongful death claim. (FAC ¶¶ 68-71, 79-98, 219-247, 248-262 [willful misconduct], 263 [incorporating prior allegations].)

A. Legal Background and Elements of Wrongful Death Claim

The elements of a cause of action for wrongful death "include (1) a 'wrongful act or neglect' on the part of one or more persons that (2) 'cause[s]' (3) the 'death of [another] person' [citation omitted] – on legal theories of negligence and strict liability." (*Norgart v. Upjohn Co.* (1999) 21 Cal.4th 383, 390.) (*See also Quiroz v. Seventh Ave. Center* (2006) 140 Cal.App.4th 1256, 1263; Code Civ. Proc. § 377.60.) The "wrongful act" on which Plaintiff relies to support this cause of action is not negligence, but willful misconduct or recklessness based on the factual allegations of the second cause of action for elder abuse. (FAC ¶¶ 68-71, 79-98, 219-247, 248-262 [willful misconduct], 263 [incorporating prior allegations].)

The Elder Abuse Act generally addresses protection of the elderly, defined as persons 65 or older, and dependent adults from egregious acts and omissions committed by those in a caregiver or custodial

care relationship with the elderly. Neglect for purposes of the Elder Abuse Act is defined in Welfare & Institutions Code section 15610.57 and includes "[f]ailure to protect from health and safety hazards." (Welf. & Inst. Code § 15610.57(b)(3).) A claim for neglect under Elder Abuse Act is not based on professional negligence. (*Country Villa Claremont Healthcare Center, Inc. v. Superior Court* (2004) 120 Cal.App.4th 426, 432.) "[N]eglect' within the meaning of Welfare and Institutions Code section 15610.57 covers an area of misconduct distinct from 'professional negligence.'" (*Covenant Care, Inc. v. Superior Court* (2004) 32 Cal.4th 771, 783.) "As used in the [Elder Adult and Dependent Adult Civil Protection] Act, neglect refers not to the substandard performance of medical services but, rather, to the 'failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations.'" (*Id.*) (See also *Winn v. Pioneer Medical Group, Inc.* (2016) 63 Cal.4th 148, 152, 157-160, 165 [neglect under the Elder Abuse Act does not apply to the negligent provision of medical services].)

Under *Carter v. Prime Healthcare Paradise Valley LLC* (2011) 198 Cal.App.4th 396, Plaintiff must plead and prove "facts establishing that the defendant: (1) had responsibility for meeting the basic needs of the elder or dependent adult, such as nutrition, hydration, hygiene or medical care [citations]; (2) knew of conditions that made the elder or dependent adult unable to provide for his or her own basic needs [citations]; and (3) denied or withheld goods or services necessary to meet the elder or dependent adult's basic needs, either with knowledge that injury was substantially certain to befall the elder or dependent adult (if the plaintiff alleges oppression, fraud or malice) or with conscious disregard of the high probability of such injury (if the plaintiff alleges recklessness) [citations]." (*Id.* at 406-407 [emphasis added].) "To recover the enhanced remedies available under the Elder Abuse Act from a health care provider, a plaintiff must prove more than simple or even gross negligence in the provider's care or custody of the elder. [Citations omitted.]" (*Id.* at 405.) Under Welfare & Institutions Code section 15657, to obtain the enhanced remedies provided in that statute, Plaintiff must prove by clear and convincing evidence both that there was "neglect" as defined in the statute and "that the defendant has been guilty of recklessness, oppression, fraud, or malice in the commission of this abuse." (Welf. & Inst. Code § 15657.) (See also *Covenant Care, Inc. v. Superior Court, supra*, 32 Cal.4th at 783.) "Recklessness involves ' "deliberate disregard" of the "high degree of probability" that an injury will occur' and 'rises to the level of a "conscious choice of a course of action . . . with knowledge of the serious danger to others involved in it." ' [Citation omitted.]" (*Id.* at 405.)

There is no dispute that GHC had custodial care of Ms. Marshall, and that she was bedridden and incapable of providing for her own basic life needs. The dispute centers on whether GHC and Life Generations "denied or withheld goods or services necessary to meet the elder or dependent adult's basic needs, either with knowledge that injury was substantially certain to befall the elder or dependent adult (if the plaintiff alleges oppression, fraud or malice) or with conscious disregard of the high probability of such injury (if the plaintiff alleges recklessness) [citations]." (*Id.* at 406-407.) GHC and Life Generations dispute whether any acts or omissions by them caused Ms. Marshall's death.

Plaintiff contends that GHC and Life Generations committed neglect by denying or withholding care necessary to meet Plaintiff's basic needs in light of her high risk of pressure injuries and skin injuries from her incontinence and that GHC and Life Generations did so "recklessly," consisting of a " 'deliberate disregard' of the 'high probability' that an injury will occur, or a 'conscious choice of a

course of action with knowledge of the serious danger to others involved in it.' (*Delaney v. Baker* (1999) 20 Cal.4th 23, 31-32.)" (Opp. p. 22, ll. 17-20.) She contends that the neglect at GHC and Life Generations was a substantial factor causing her death more than eight months after she left their care.

B. Undisputed Material Facts

A number of the facts are undisputed. Ms. Marshall was 85 years old when she was first admitted to Walnut Creek Nursing & Rehabilitation Center (aka GHC of Walnut Creek), a licensed skilled nursing facility, on January 27, 2021. (UMF 8, 11.) The duration of Ms. Marshall's time at GHC is undisputed, specifically she was a resident at GHC Walnut Creek from January 27, 2021 to February 9, 2021 (14 days), and from February 13, 2021 to March 20, 2021 (35 days), based on multiple UMFs addressed below.

When she was admitted on January 27, 2021, Ms. Marshall was noted to be at high risk for pressure ulcers and had a medical history that included a stroke suffered four and a half months prior to her admission, chronic respiratory and heart failure, hypertension, atrial fibrillation, neuropathy, edema, asthma and several other conditions, including peripheral vascular disease. (UMF 11, 12.) She was "bedbound with limited functionality status/post tracheostomy and PEG," and lacking capacity to make decisions. (UMF 11.)

On her admission to GHC, Ms. Marshall had an unstageable pressure ulcer injury on her right outer ankle, a wound on her right outer ankle, and a black callus on her left fifth toe. (UMF 17.) On the date of her admission, there were orders placed to "float" Ms. Marshall's heels, to turn and reposition Ms. Marshall, and to monitor for incontinence every two hours, in addition to other skin monitoring for pressure injury risks. (UMF 14.) There was also a plan for wound care for Ms. Marshall's right ankle pressure injury and left fifth toe. (UMFs 15, 16, 18.) She received a number of evaluations on January 28, 2021, including evaluations by physical therapy, occupational therapy, and nutrition services. (UMFs 20-22.) The next day she was evaluated by speech therapy. (UMF 25.)

On February 3, 2021, there was a plan to treat Ms. Marshall's ankle wound which included, among other things, cleansing the wound with theraworx, covering the wound with foam, and continuing to monitor the wound for healing, a plan to treat her moisture-associated skin damage (MASD) to her bilateral buttocks by application of calazime and A&D ointment, and a plan to treat her left fifth toe callus with skin prep and leaving the toe open to the air. (UMFs 33-35.) On February 9, 2021, Ms. Marshall had elevated sodium levels, and a doctor ordered that she be taken to the John Muir Hospital emergency department. (UMF 42.)

It is undisputed that during her stay at GHC Walnut Creek between January 27, 2021 and February 9, 2021, she had weekly skin checks, pressure ulcer BWAT reports, and skin alteration reports, and a weekly Braden Scale pressure ulcer risk assessment. (UMFs 43, 44.) She was also seen by a doctor on seven dates during that 14 day time period. (UMF 45.)

On February 13, 2021, Ms. Marshall was readmitted to GHC Walnut Creek. (UMF 46.) GHC had a care plan for Ms. Marshall's right ankle wound care, bilateral buttock MASD, and left fifth toe callus similar to the prior plans. (UMFs 49-51.) As of February 13, 2021 when she was readmitted, her right ankle, lower fibula was measured at 2.0x0.5x0.4, her buttock MASD with an open area of 4.0x1.0 (on both sides, or the left side measured at 4.0x1.5 cm), and the left fifth toe at 1.5x1.5 cm. (UMF 48 and Pl.

Resp. to UMF 48.) On February 17, 2021, there was no change in Ms. Marshall's wounds and wound care plan from the prior week. (UMF 57.) Ms. Marshall's wounds remained stable in size based on the nursing documentation on February 13, February 17, and February 24, 2021. (Pl. Resp. to UMF 65.)

On March 3, 2021, a new deep tissue, unstageable injury to Ms. Marshall's left heel was noted that was covered in black eschar and measured 9.0x6.0 cm, and the care plan was to continue to keep her heels floated at all times and apply betadine to the wound twice a day. (UMF 74.) On March 4, 2021, an arterial ultrasound of her left leg was ordered. (UMF 76.) On March 9, 2021, moving parties contend that Ms. Marshall's left heel wound was "noted to be vascular, not pressure-related after a venous ultrasound showed peripheral artery disease," the cause of which Plaintiff disputes. (UMF 85; Pl. Resp. to UMF 85.) On March 10, 2021, Ms. Marshall's right ankle wound was 6.0x4.0 cm and covered in black eschar. She was seen by a wound specialist on March 13, 2021, and she had a vascular surgery consult on March 15, 2021. (UMFs 92, 95.) On March 17, 2021, Ms. Marshall's right ankle wound was measured at 15.5x9.0 (or 15.5x9.5) cm and 90% covered in black eschar. (UMF 98; Pl. Resp. to UMF 98.) On March 17, 2021, her left heel peripheral arterial vascular ulcer measured 8.5x5.0 cm and was 100% covered in black eschar, and she was seen by a wound specialist and vascular specialist. (UMF 99.) On the same date, her buttocks MASD had redness but no signs of infection and an open area of 1.0x5.0x0.1. (UMF 100.)

On March 20, 2021, Ms. Marshall had a fever of 103.4, and her doctor ordered her to be transferred to the emergency department and was sent to John Muir Medical Center Walnut Creek. (UMFs 105, 106.) Ms. Marshall undisputedly had Braden Scale pressure sore assessments on February 13, 2021, March 5, 2021, and March 12, 2021, but Plaintiff disputes that she had such assessments on February 14, 2021, February 19, 2021, and February 26, 2021 based on the records cited and provided in support of the motion. (UMF 107; Pl. Resp. to UMF 107.) It is undisputed she had weekly skin checks, pressure Ulcer BWAT reports, and skin alteration reports on six dates between February 13, 2021 and March 17, 2021. (UMF 108.)

C. UMFs Disputed by Plaintiff

Plaintiff disputes a number of the UMFs in which GHC and Life Generations state that Ms. Marshall was turned and repositioned "according to the treatment plan" on particular dates. Plaintiff also disputes a number of the UMFs in which GHC and Life Generations state that wound care was provided to Ms. Marshall in accordance with the care plan on particular dates. The basis for Plaintiff's dispute in most instances is that the evidence offered by the moving parties in support of those statements, specific medical records from GHC for specific dates, do not show the facts stated, specifically that Ms. Marshall was turned and repositioned and/or given a particular type of wound care on the date in question or in accordance with the treatment plan. (See, e.g., UMFs 19 [recommended treatment for wounds, not that treatment was provided], 26, 29, 31, 38, 39, 40, 41, 52, 59, 67 [pp. 783 and 785 describe that wound care was provided on 2/23/2021 and 2/24/2021, not 2/26/2021 as stated in the UMF], 81; Pl. Resp. to UMFs 19, 26, 29, 31, 38, 39, 40, 41, 52, 59, 67, 81.)

Though Plaintiff contests whether the GHC medical records cited and submitted in support of the foregoing UMFs support the facts stated regarding turning and repositioning in particular on certain dates, Plaintiff has conceded through her expert declaration the fact that the GHC medical records show consistent turning and repositioning of Ms. Marshall regularly every two hours from January 27,

2021 through February 9, 2021, and again from February 13, 2021 through March 5, 2021, and on March 8, 9, 11, 12, 13, 14, 16, 17, and during the six-hour period on March 20, 2021 before she was taken to the hospital. (Bain Decl. Exh. B.) Plaintiff has genuinely disputed five UMFs raising triable issues as to whether GHC failed to turn and reposition Ms. Marshall every two hours pursuant to her care plan on March 6, 7, 10, 15, and 19. (UMFs 79, 80, 86, 94, and 103; Pl. Resp. to UMFs 79, 80, 86, 94, and 103 and Bain Decl. ¶ 27.) While Plaintiff through Dr. Bain attempts to dispute the veracity of the entries in the records, Dr. Bain's declaration testimony admits that the records show what these UMFs state; namely, that Ms. Marshall was regularly turned and repositioned every two hours pursuant to the care plan on the above-cited dates.

In addition to stating turning and repositioning occurred pursuant to the care plan, UMFs 19, 26, 29, 31, 38, 39, 40, 41, 52, 59, 67, and 81 specifically state wound care "was completed according to the treatment plan" on certain specific dates, with citations to specific pages of the GHC medical records as support. The cited pages of the medical records do not refer to "wound care" such that these UMFs are not supported by the evidence, as pointed out by Plaintiff. (See Pl. Resp. to UMFs 19, 26, 29, 31, 38, 39, 40, 41, 52, 59, 67, 81.) If the evidence submitted by the moving parties does not support the fact stated in those UMFs, then moving parties have not met their initial burden as to those facts, such that moving parties have not shown that wound care was provided pursuant to the treatment plan on January 27, January 30, February 2, February 3, February 6, February 7, February 8, February 9, February 14, February 19, February 26, and March 8, 2021. (UMFs 19, 26, 29, 31, 38, 39, 40, 41, 52, 59, 67, and 81; Pl. Resp. to UMFs 19, 26, 29, 31, 38, 39, 40, 41, 52, 59, 67, 81.) Though Plaintiff disputes a number of other UMFs on the ground that the particular page cited by moving parties as the evidentiary support for the statement regarding wound care does not refer to wound care or does not apply to the date stated in the UMF, the forgoing list of genuinely disputed UMFs excludes UMFs if the Court found wound care, skin care, or wound or skin treatment on an immediately preceding page in the records submitted in Exhibit G to the Weller Declaration.

Plaintiff has also genuinely disputed UMF 107. The records cited in some instances are not provided in Exhibit G. Based on the Court's review of Exhibit G, there were no Braden weekly assessments in the records showing assessments were made on February 14 or February 26 as stated in the UMF. (See UMF 107; Pl. Resp. to UMF 107.)

The existence of disputed UMFs does not compel denial of a motion for summary judgment, though they can support denial of the motion. (*Compare Nazir v. United Airlines, Inc.* (2009) 178 Cal.App.4th 243, 252 ["[I]f a triable issue is raised as to any of the facts in your separate statement, the motion must be denied! [Citations, internal quotation marks omitted.]"] and *Insalaco v. Hope Lutheran Church of West Contra Costa County* (2020) 49 Cal.App.5th 506, 521 [holding that a party moving for summary judgment who relies on facts that it designated undisputed to argue in its favor on summary judgment cannot then deem those same facts immaterial when they are disputed by the opposing party] to *Pereda v. Atos Jiu Jitsu LLC* (2022) 85 Cal. App. 5th 759, 773 [finding motion for summary judgment motion properly granted even though plaintiff disputed 54 out of 63 material facts listed in defendants' separate statement.].)

UMFs 110-124 address whether any acts or omissions of GHC as a skilled nursing facility, including its nursing staff, breached the applicable standard of care, whether any of its acts or omissions are sufficient to constitute recklessness or malice, whether any acts or omissions of GHC were a

substantial factor in the cause of Ms. Marshall's death, and whether any acts or omissions of an employee on which enhanced remedies could be awarded were authorized or ratified by the employer GHC or Life Generations. The Court will address those UMFs and the extent to which they raise triable issues of material fact below in the context of applying the law on elder neglect to the related facts and evidence.

D. Application of the Law to the Case and Existence of Triable Issues

1. Significant Pattern of Withholding of Care Shown by Clear and Convincing Evidence

To rise to the level of elder abuse or neglect under the Elder Abuse Act, more than mere negligence or an inadvertent failure to provide care is needed. The standard for recklessness under *Delaney* and *Covenant Care* is set forth above; the acts or omissions must rise to a level of conscious choice of a course of conduct with knowledge of serious danger posed to the patient. (*Delaney, supra*, 20 Cal. 4th at 31-32; *Covenant Care, supra*, 32 Cal.4th at 783.) The Court is also cognizant of the standard of proof of clear and convincing evidence, a standard Plaintiff does not contest in the opposition applies to elder abuse. (*Delaney, supra*, 20 Cal.4th at 31 ["In order to obtain the remedies available in section 15657, a plaintiff must demonstrate by clear and convincing evidence that defendant is guilty of something more than negligence; he or she must show reckless, oppressive, fraudulent, or malicious conduct." (Emphasis added.)]; *Basich v. Allstate Ins. Co.* (2001) 87 Cal.App.4th 1112, 1117-1118 [affirming grant of summary adjudication of claim for punitive damages, stating "If, in fact, the trial court viewed the evidence submitted by Basich in opposition to the motion for summary adjudication with the clear and convincing evidence standard in mind, it did so properly. [Citation omitted.]"].)

Plaintiff cites and relies on *Sababin v. Superior Court* (2006) 144 Cal.App.4th 81. The Court of Appeal in *Sababin* reversed a trial court order granting summary adjudication of an Elder Abuse Act claim. The trial court concluded that the facts showed nothing more than negligence, but the Court of Appeal disagreed, finding that the care center's failure to follow the established care plan for monitoring and addressing potential pressure ulcers raised triable issues of fact regarding whether the plaintiff could establish neglect under the Elder Abuse Act. (*Id.* at 83-87, 90 [in which appellant's challenged whether "there are triable issues of act as to whether Covina's employees acted with recklessness, oppression or malice when they failed to follow the care plan Covina established for maintaining the health of Renteria's skin"].) The Court in *Sababin* rejected the defendant's argument that if some care had been provided, the Court could not find abuse or neglect. "If some care is provided, that will not necessarily absolve a care facility of dependent abuse liability. For example, if a care facility knows it must provide a certain type of care on a daily basis but provides that care sporadically or is supposed to provide multiple types of care but only provides some of those types of care, withholding of care has occurred. In those cases, the trier of fact must determine whether there is a significant pattern of withholding portions or types of care. A significant pattern is one that involves repeated withholding of care and leads to the conclusion that the pattern was the result of choice or deliberate indifference." (*Id.* at 90 [emphasis added].)

The facts and evidence in *Sababin* was disputed. The Court summarized the facts relevant to its conclusion there were triable issues: "According to the Department's April 28, 2003 survey, Renteria's December 19, 2002 care plan 'indicated that staff would monitor skin daily for redness or breakdown

and report to the physician for a treatment order.' It was noted that on admission to East Valley, Renteria 'had lacerations on her toes and feet with poor skin condition on both buttocks' and that a nurse stated that 'the buttocks area was dark and red and felt squishy.' Also, her heel was bruised and had areas of redness. Despite the foregoing, Covina did not have any documentation of Renteria's skin condition. When Covina's treatment nurse was interviewed, she stated that she had not done a skin condition report for Renteria and was not aware of Renteria's reddened buttocks. A review of Covina's records indicated that there was no evidence of Renteria's right foot condition or reddened buttocks. Renteria's medical chart revealed that no one at Covina notified a physician of the need for a treatment order." (*Id.* at 89-90.) With acknowledgment of the "significant pattern" required to support the elder neglect claim, the Court, relying on the rule of liberal construction of the evidence in opposition to a motion for summary judgment, held, "{W}hen the evidence and inferences are liberally construed, we easily conclude that there is a triable issue as to whether Covina's employees acted with recklessness, oppression or malice. A trier of fact could find that when a care facility's employees ignore a care plan and fail to check the skin condition of a resident with Huntington's Chorea, such conduct shows deliberate disregard of the high degree of probability that she will suffer injury. Though Covina offers conflicting declarations regarding the care provided, we are not permitted to weigh the evidence." (*Sababin, supra*, 144 Cal.App.4th at 90 [emphasis added].)

When Ms. Marshall was first admitted to GHC Walnut Creek on January 27, 2021, she had existing injuries that were documented: an unstageable pressure ulcer (1.0x0.5 cm) on her right outer ankle, a wound (2.0x1.5x0.4) on her right outer ankle, and a black callus on her left fifth toe, and MASD was noted on her right buttock. (UMF 17; Bain Decl. ¶ 10.) On February 3, 2021, the MASD was noted as healing. (Bain Decl. ¶ 14.) When she was readmitted to GHC Walnut Creek on February 13, 2021 after being sent to the hospital on February 9, 2021, she had what Bain characterizes as "chronic trauma" in her sacrum/buttocks area from MASD measured at 4.0x1.5 cm on the left and 4.0x1.0 cm, an unstageable right ankle pressure ulcer measured at 2.0x0.5x0.4 cm, and the fifth left toe callous measuring 1.5x1.5 cm. (Bain Decl. ¶ 20.) Though the ankle and toe injuries existed when she was admitted, the MASD clearly expanded and was exacerbated by February 9, 2021 during GHC's care.

By March 20, 2021 when she was discharged from GHC Walnut Creek to the hospital for the second and last time, she had developed a left heel deep tissue injury measuring 9.0x6.0 cm on March 3 that had increased in size to 8.5x5.0 cm on the discharge date, and the unstageable right ankle pressure injury had increased to 15.5x9.0 cm with black eschar covering 90% of the wound. (Bain Decl. ¶¶ 24-26.) When she was discharged from GHC Walnut Creek and admitted to the hospital on March 20, 2026, she was in critical condition and diagnosed with severe sepsis suspected to be from her lower extremity ulcers, which included the left heel ulcer and infected right leg, and had a sacrococcygeal wound that extended to her right buttock. (Bain Decl. ¶ 32.) Between March 6, 2026 and March 20, 2026 (15 days), the period in which the pressure ulcer wounds worsened, there were five days on which GHC's records show the required two-hour turns and repositioning were not documented as having been made, including for time periods that extended for several hours or even an entire shift. (Bain Decl. ¶ 27 and Exh. B.) Dr. Bain calculates that GHC made 82.56% of the turns in that period but **missed 17.44%** of the two-hour turns provided for in the care plan. (Bain Decl. ¶ 27.) As set forth above, the UMFs are unsupported or genuinely disputed as to whether Ms. Marshall received wound care in accordance with the treatment plan on 12 dates cited above (see discussion of disputed UMFs 19, 26, 29, 31, 38, 39, 40, 41, 52, 59, 67, 81 above) of the 49 total days in which she was admitted to

GHC Walnut Creek, **roughly 24%** of the days she was admitted.

The Court concludes that Plaintiff has raised triable issues of fact as to whether GHC failed to follow its turning and repositioning and wound care plans for Ms. Marshall (see discussion of disputed UMFs 19, 26, 29, 31, 38, 39, 40, 41, 52, 59, 67, 79, 80, 81 86, 94, 103, and 107 above). Plaintiff has raised triable issues as to whether a trier of fact could find by clear and convincing evidence, based on the UMFs unsupported by the evidence, the disputed facts, and the admissible expert evidence from Dr. Bain, that Plaintiff has shown those failures constitute recklessness in the breach of the applicable standard of care for a skilled nursing facility and its nursing staff (See *also* Rulings on Evidentiary Objections to Bain Declaration below.) The facts show that GHC was clearly aware of the significance of the turning and repositioning of the patient and her wound care as GHC developed specific care and treatment plans to address those conditions. Questions of fact exist as to whether, despite that knowledge and the existence of the care plan, GHC failed to abide by those components of the care plan on multiple occasions as indicated above. The Court finds UMFs 113 [MASD on buttocks], 115, 116, 117 [genuinely disputed that MASD on buttocks and skin tear on right thigh originated at GHC], 119 [wound care and continued MASD resulted from failure to meet standard of care], 120 [documentation of turning and repositioning shows 5 days/17.44% of turns missed], 122, and 124 are genuinely disputed; they raise triable issues regarding whether Ms. Marshall suffered neglect rising to the level of recklessness based on GHC's failure to follow her care plan (or lack of evidence by the moving parties demonstrated they followed the care plan) regarding turning and repositioning and wound care.

The Court also notes that the issues on a motion for summary judgment are bounded by the pleadings. The issues to be considered on a motion for summary judgment or summary adjudication are defined by the pleadings. (*Anderson v. Fitness Internat., Inc.* (2016) 4 Cal.App.5th 867, 876; *Teselle v. McLoughlin* (2009) 173 Cal.App.4th 156, 161 ["The complaint measures the materiality of the facts tendered in a defendant's challenge to the plaintiff's cause of action."].) Plaintiff has alleged understaffing and lack of supervision by the care facilities in which Ms. Marshall was admitted, including failure to meet the minimum staffing levels required under the Health & Safety Code. (FAC ¶¶ 219-230.) The moving papers do not address the adequacy or inadequacy of the staffing levels at GHC. An inference could be drawn from the 17.44% of missed turns in the two-week period in March 2021 and the lack of evidence documenting wound care on 24% of the days of Ms. Marshall's admission that GHC may have been understaffed which may further support a finding of recklessness, a fact alleged and not negated in the motion.

2. Causation

Plaintiff cites *Espinosa v. Little Co. of Mary Hospital* (1995) 31 Cal.App.4th 1304 on the issue of causation, a case which addressed causation in the context of a medical malpractice claim. "In a medical malpractice action the element of causation is satisfied when a plaintiff produces sufficient evidence 'to allow the jury to infer that in the absence of the defendant's negligence, there was a reasonable medical probability the plaintiff would have obtained a better result. [Citations.]' [Citation omitted.]" (*Espinosa v. Little Co. of Mary Hospital* (1995) 31 Cal.App.4th 1304, 1314-1315 [quoting *Alef v. Alta Bates Hospital* (1992) 5 Cal.App.4th 208, 216].) The defendant's negligence must be a "cause-in-fact" of the injury, and "liability attaches if negligence is a substantial factor in causing the injury." (*Jennings v. Palomar Pomerado Health Systems, Inc.* (2003) 114 Cal.App.4th 1108, 1118 and

fn. 3.)

The Court in *Espinosa v. Little Co. of Mary Hospital*, *supra*, addressed the effect of multiple factors potentially causing the harm in that case. "Given this testimony, that defendants' conduct was a substantial factor in bringing about the outcome, Dr. Gabriel's inability to pin down the exact extent to which defendants' conduct contributed to the outcome is immaterial for purposes of causation. Clearly, where a defendant's negligence is a concurring cause of an injury, the law regards it as a legal cause of the injury, regardless of the extent to which it contributes to the injury. [Citation omitted.]" (*Espinosa v. Little Co. of Mary Hospital*, *supra*, 31 Cal.App.4th at 1317-1318.) The Court in that case noted that the expert did not testify that the negligent conduct was a "possible" cause but that the conduct was a substantial factor causing the brain damage at issue in that case. (*Id.* at 1318.)

UMFs 110, 112 [causation of heel injury developed at GHC], and 114 address causation. The Court concludes Dr. Bain's declaration adequately disputes that acts and omissions of GHC which resulted in the MASD and pressure injuries to develop or progress may have been a "substantial factor" in causing Plaintiff's death. Dr. Bain offers reasoned explanations for his opinions supported by the systemic review and meta-analysis described in his declaration relating pressure ulcers to increased rates of mortality. The Court cannot weigh the conflicting evidence. The evidence could support a finding that the neglect was a "substantial factor" contributing to her death.

3. Employer Ratification Issue

Welfare and Institutions Code section 15657(c) provides: "The standards set forth in subdivision (b) of Section 3294 of the Civil Code regarding the imposition of punitive damages on an employer based upon the acts of an employee shall be satisfied before any damages or attorney's fees permitted under this section may be imposed against an employer." Civil Code section 3294(b) states: "An employer shall not be liable for damages pursuant to subdivision (a), based upon acts of an employee of the employer, unless the employer had advance knowledge of the unfitness of the employee and employed him or her with a conscious disregard of the rights or safety of others or authorized or ratified the wrongful conduct for which the damages are awarded or was personally guilty of oppression, fraud, or malice. With respect to a corporate employer, the advance knowledge and conscious disregard, authorization, ratification or act of oppression, fraud, or malice must be on the part of an officer, director, or managing agent of the corporation." (Emphasis added.)

Civil Code section 3294(b) was enacted decades before statutes authorizing the creation of limited liability companies were enacted. (See Fomer Beverly-Killea Limited Liability Company Act (Beverly-Killea Act; Corp. Code former § 17000 *et seq.* enacted in 1994, now replaced with California Revised Uniform Limited Liability Company Act, Corp. Code § 17701.01 *et seq.* enacted in 2014; *Western Surety Co. v. La Cumbre Office Partners, LLC* (2017) 8 Cal. App. 5th 125, 131.) Courts have explained that " '[a] limited liability company is a hybrid business entity formed under the Corporations Code and consisting of at least two 'members' [citation] who own membership interests [citation]. The company has a legal existence separate from its members. Its form provides members with limited liability to the same extent enjoyed by corporate shareholders [citation] . . . ' [Citations, internal quotation marks omitted.]" (*Western Surety Co.*, *supra*, 8 Cal. App. 5th at 131 [quoting *PacLink Communications Internat., Inc. v. Superior Court* (2001) 90 Cal.App.4th 958, 963].) There is at least one recent decision in which the Court appears to have assumed, though without deciding, that the

provisions of Civil Code section 3294(b) apply to a limited liability company. (*Samantha B. v. Aurora Vista Del Mar, LLC* (2022) 77 Cal. App. 5th 85, 105-106.)

Moving parties include a single UMF (UMF 123) which repeats a statement in the Dennin Declaration that it is her opinion "that there is no evidence that any officer, director, or managing agent of [GHC or Life Generations] had knowledge of, directed, or ratified any conduct that could constitute elder abuse or willful or reckless misconduct." (Dennin Decl. ¶ 11.) Ms. Dennin attests that she reviewed both the GHC chart pertaining to Ms. Marshall and GHC's "Nursing Service Policies and Procedures Manual – 2018, 2021, 2022, including Operational Policy and Procedures Manuals for 2021" in formulating her opinions, including the opinion stated in paragraph 11 of her declaration. (Dennin Decl. ¶ 5.)

The only reasonable inference from UMF 123 is that Ms. Dennin's review of the GHC records and applicable procedure manuals does not indicate any person of managerial-level authority for the employer authorized or ratified any failures of care by the nurses and other providing Ms. Marshall's care. Plaintiff submits no evidence to contest that facts stated in this UMF, and the reasonable inference from the fact stated is that the "employer" did not ratify or authorize any employee's conduct on which the award of enhanced remedies under the Elder Abuse Act could be based.

The Court rejects Plaintiff's argument that moving parties failed to adequately raise this deficiency in Plaintiff's ability to prove its claims against them. Plaintiff concedes that moving parties raised the lack of evidence of employer ratification or authorization of the employees' conduct in their initial memorandum sufficiently such that Plaintiff responded to the argument in her opposition. (Opp. p. 26, ll. 1-25.) Moving parties included UMF 123.

Nevertheless, the Court concludes that moving parties failed to meet their initial burden on this issue in the moving papers by failing to address the allegations in the FAC of a pattern of understaffing and noncompliance with the staffing requirements of Health & Safety Code section 1295, which Plaintiff alleged resulted in omissions in the compliance with her care and treatment plans. (See, e.g., FAC ¶ 220 ["Defendants routinely failed to staff sufficiently so that needed treatment was *not* performed or adequately/timely performed." (Italics in original.)].) (See *also* generally FAC ¶¶ 219-247 [corporate-level decisions on budgets and inadequate staffing levels].) While UMF 123 and Ms. Dennin's declaration may state that she did not observe evidence of knowledge, direction, or ratification of "any conduct" that could constitute elder neglect or recklessness by a member of the nursing staff, strictly construed, that evidence does not address the employer management's knowledge, direction, or ratification of limitations in staffing levels or staffing levels not in compliance with the Health & Safety Code which may have resulted in the omissions in care. For these reasons, the Court finds summary judgment cannot be granted for moving parties based on lack of employer authorization or ratification of the neglect.

Moving Parties' Evidentiary Objections to Bain Declaration

Obj. Nos. 1 and 2 - **Overruled.** The declarant sets forth a factual basis for his opinion that charting showing perfectly timed two-hour turning and repositioning is "clinically implausible" based on the remainder of paragraph 28 in which Dr. Bain explains that a patient such as Ms. Marshall typically is subject to multiple therapies, treatments and interventions that "necessarily create variability in timing and work flow," making the "complete absence of any variation" a concern regarding accuracy

and reliability of the charting regarding the turning and repositioning. The Court agrees that whether the evidence is reliable or accurate will be for a trier of fact to determine.

Obj. No. 3 – **Sustained.**

Obj. Nos. 4, 5 - **Overruled.** As set forth above, the evidence in the moving papers and GHC's medical records reviewed by Dr. Bain show missed turning/repositioning which supports the opinion that the development and progression" of Ms. Marshall's injuries are consistent with inadequate pressure relief and turning. The Court cannot weigh the credibility of the opinion but can only assess whether there is a reasoned explanation for the opinion. (*Fernandez v. Alexander* (2019) 31 Cal.App.5th 770, 781-782; *Sanchez v. Kern Emergency Medical Transportation Corp.* (2017) 8 Cal.App.5th 146, 155-156 [among other things, to be admitted, the medical expert opinion cannot be conclusory, must contain a "reasoned explanation" for the opinions expressed, and with respect to a causation opinion, must be sufficient to convince a trier of fact "that it is more probable than not the negligent act was a cause-in-fact of the plaintiff's injury." (*Sanchez v. Kern Emergency Medical Transportation Corp.*, *supra*, 8 Cal.App.5th at 160; *Fernandez v. Alexander*, *supra*, 31 Cal.App.5th at 781-782 (expert declaration was conclusory, not supported by a reasoned explanation, and his conclusion on causation, though stated to a "reasonable degree of medical probability," failed to connect the factual predicates to the conclusion, making the declaration inadmissible on summary judgment].) Here, Dr. Bain's declaration connects the facts regarding how pressure ulcers and injuries occur with the patient's condition and symptoms and provides a reasoned explanation for why her injuries occurred or progressed. As to the statement regarding "failure of preventive care" (Bain Decl. ¶ 36, Obj. No. 5), the Court interprets the statement based on the language used. It does not refer to breach of the standard of care but rather factually, that "preventive care" failed which resulted in the development of the heel pressure injury.

Obj. No. 6 – **Overruled.** Dr. Bain explains what MASD is, how it arises, and how it is prevented, which provides the foundation for the opinion. (See Bain Decl. ¶¶ 19, 20, 43, 44.)

Obj. No. 7 – **Overruled.** Foundation is laid in the declaration for the pressure injuries that developed and/or progressed during Ms. Marshall's periods of admissions to GHC Walnut Creek and regarding the medical literature regarding the increased risk of mortality when a patient has pressure injuries. (Bain Decl. ¶ 46.) Dr. Bain attests that he was aware of Ms. Marshall's other health conditions and comorbidities, which the medical literature cited addresses, and that he reviewed Ms. Marshall's medical records from the other facilities where she was placed in formulating his opinion. (Bain Decl. ¶¶ 8-11, 46.) The Court interprets the term "neglect" as used in the sentence objected to as referring to the common understanding of the term "neglect," based on the failures of care noted by the declarant from the GHC records discussed in the declaration, and at most, an admissible ultimate fact but not a legal conclusion. (*Towns v. Davidson* (2007) 147 Cal. App. 4th 461, 472 ["Although the expert's testimony may embrace an ultimate factual issue (Evid. Code, § 805), it may not contain legal conclusions. [Citation omitted.]" (Emphasis added.).) As such, it cites an "evidentiary fact" to which Dr. Bain as a qualified expert may attest. (*Hayman v. Block* (1986) 176 Cal. App. 3d 629, 639.) (See *also* Evid. Code § 805 ["Testimony in the form of an opinion that is otherwise admissible is not objectionable because it embraces the ultimate issue to be decided by the trier of fact."].)

Obj. No. 8 – **Sustained in part** as to "this constituted custodial neglect" as an ultimate conclusion of law. (*Towns, supra*, 147 Cal. App. 4th at 472.)

21. 9:00 AM CASE NUMBER: N24-2030
CASE NAME: MHF HOLDING VIER GMBH & CO. KG VS. KAILA MWALE
***HEARING ON MOTION IN RE: BE RELIEVED AS COUNSEL**
FILED BY: MWALE, KAILA
TENTATIVE RULING:

Larissa A. Branes and LS Carlson Law, PC (“Counsel”) filed two separate motions to be relieved as counsel on November 25, 2025 (the “Motions to be Relieved as Counsel”). The Motions to be Relieved as Counsel were set for hearing on April 13, 2026.

Background

Counsel seeks to be relieved as attorneys for defendants Kaila Mwale and Julius Mwale (“Defendant”).

The supporting declarations filed by Counsel indicates that there has been a breakdown in the attorney-client relationship due to “irreconcilable differences.” See Declarations filed November 25, 2025, ¶2. Defendants have been given notice of the Motion to be Relieved as Counsel. *Id.* at ¶3.

Analysis

The Motion to be Relieved as Counsel is unopposed.

Disposition

The Court finds and rules as follows:

1. The Motion to be Relieved as Counsel is GRANTED.
2. The Court shall issue the order after hearing.

22. 9:00 AM CASE NUMBER: N24-2030
CASE NAME: MHF HOLDING VIER GMBH & CO. KG VS. KAILA MWALE
***HEARING ON MOTION IN RE: BE RELIEVED AS COUNSEL**
FILED BY: MWALE, JULIUS
TENTATIVE RULING:

See LINE 21 above.